

The Never-Asked: Understanding the Gap in Emotional Health Support for People Living with Diabetes in Australia

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Three in four Australians with diabetes have never been asked about their emotional health.

The people who need it most are no more likely to report being asked.

73%

of 1,092 adults living with diabetes had never been asked about their emotional health by a diabetes health professional (n = 797).

1 BACKGROUND AND AIM

Diabetes distress affects about one third of adults living with diabetes and is associated with reduced engagement with treatment, more complications and higher mortality.

Australian and international guidelines recommend that health professionals address the emotional side of diabetes as part of routine care. In 2024, Australian GPs reported screening 11.0% of people with type 1 and 6.7% of those with type 2 diabetes.

We asked people living with diabetes whether they had ever been asked.

2 METHOD

National online cross-sectional survey of Australian adults with self-reported diabetes, recruited through a research panel and paid social media. Of 1,300 responses, 1,092 were eligible.

Participants were asked: “Has your diabetes healthcare professional ever asked you about your emotional health related to living with diabetes?”

Diabetes distress was measured with the PAID-5 (score ≥ 8 indicates elevated distress). A pre-specified multivariable logistic regression identified predictors of never being asked. 533 free-text responses were analysed thematically.

Participants: median age 57 years (IQR 44–67), 62% women, 25% type 1 or LADA, 74% type 2, 40% using insulin, 36% using CGM, 70% GP-led care.

IN THEIR OWN WORDS

Seven themes from 533 free-text responses.

“Care is very black and white. If my levels are stable, no further questions are asked”

31-year-old with type 2 diabetes, diagnosed 3 years

“I wish they did coz it gets you down a lot of the time living with this”

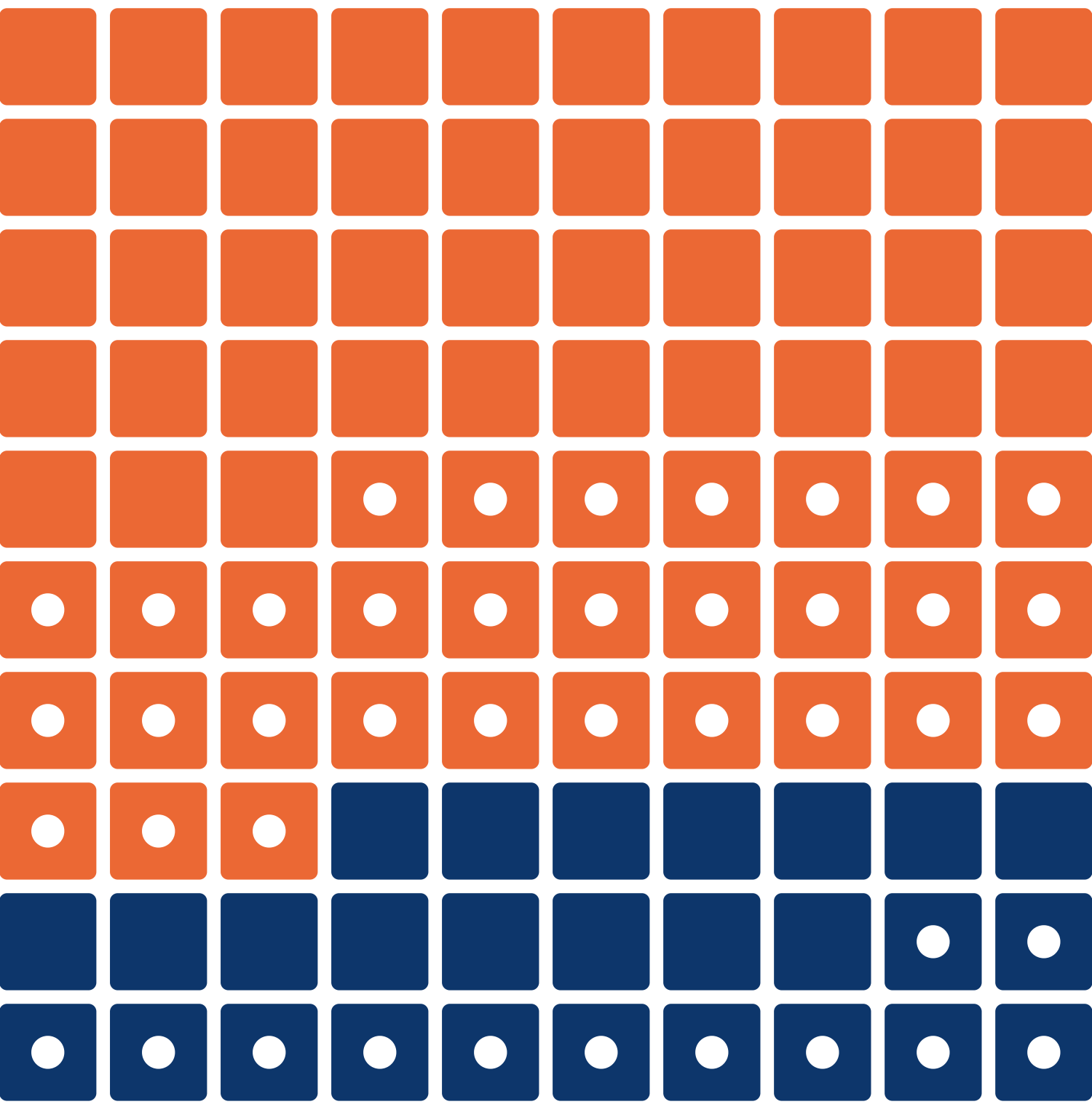
29-year-old with type 1 diabetes, diagnosed 1 year

“It made me feel that my overall well-being, not just my blood sugar, was being taken into account”

65-year-old with type 1 diabetes, diagnosed 17 years

3 RESULTS

Each square is about 1% of the 1,092 participants



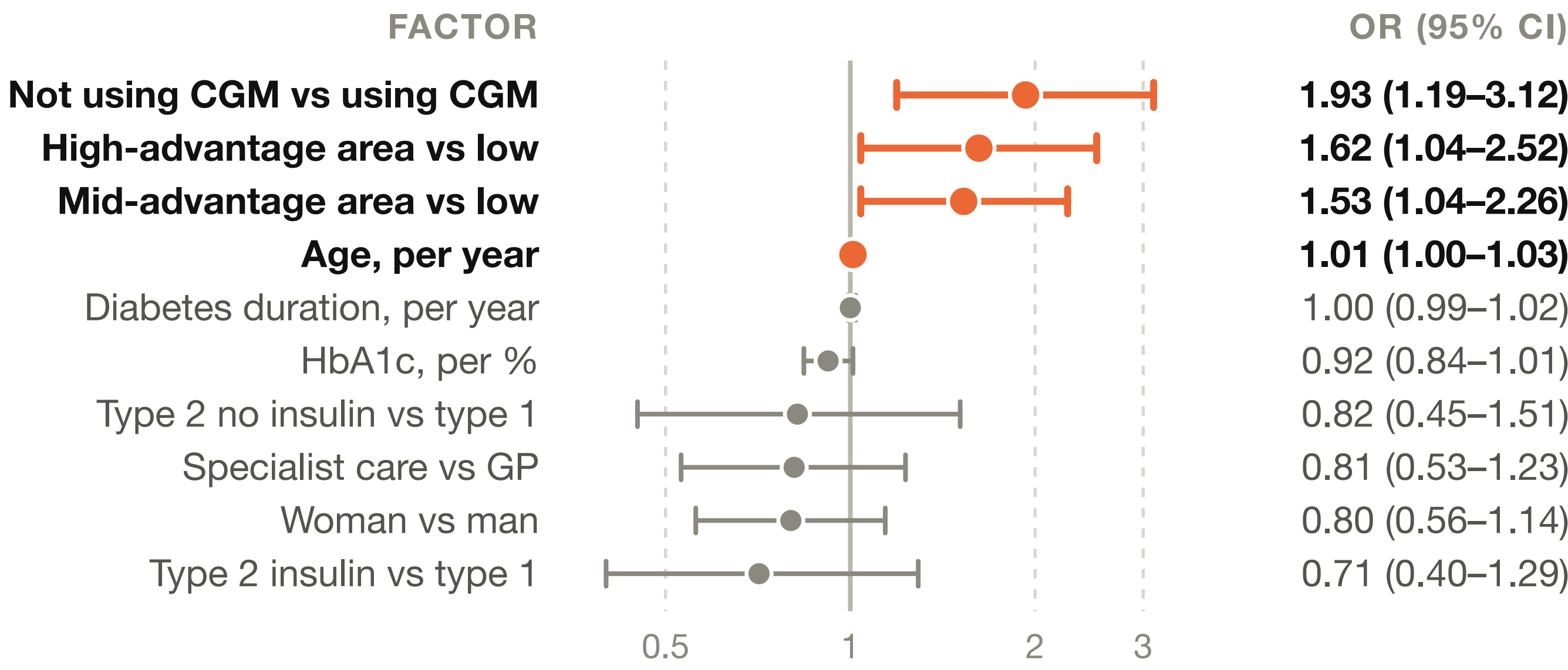
Never asked (n = 797) Asked (n = 295) Elevated distress (PAID-5 ≥ 8)

Elevated distress was as common among those never asked (41%) as among those asked (44%); p = 0.335.

- Of the 459 people with elevated distress, 328 had never been asked and only 11.5% were receiving psychological support.
- 33.7% of those never asked were neutral or dissatisfied with their emotional support, vs 13.6% of those asked (p < 0.001).

The gap reaches everyone

Adjusted odds of never being asked. No difference by diabetes type, insulin use, who leads the care, gender, diabetes duration or HbA1c.



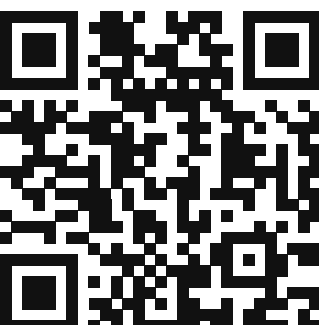
Pre-specified multivariable logistic regression (n = 738 complete cases). OR > 1 = higher odds of never being asked; orange = statistically significant. Other diabetes types (n = 7) excluded. CGM users may simply be in more frequent contact with diabetes services.

4 CONCLUSION

- Most Australians with diabetes have never had this conversation.
- Enquiry is not reaching the people who need it most.
- When asking happens, it is almost always informal (268 of 295).
- Systemic change is needed to make asking part of routine care.

LIMITATIONS

Cross-sectional and self-reported; “ever asked” relies on recall and counts a single past occasion. Convenience sample recruited through a paid panel and social media, so estimates may not generalise; type 1 diabetes is over-represented relative to the national population.



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